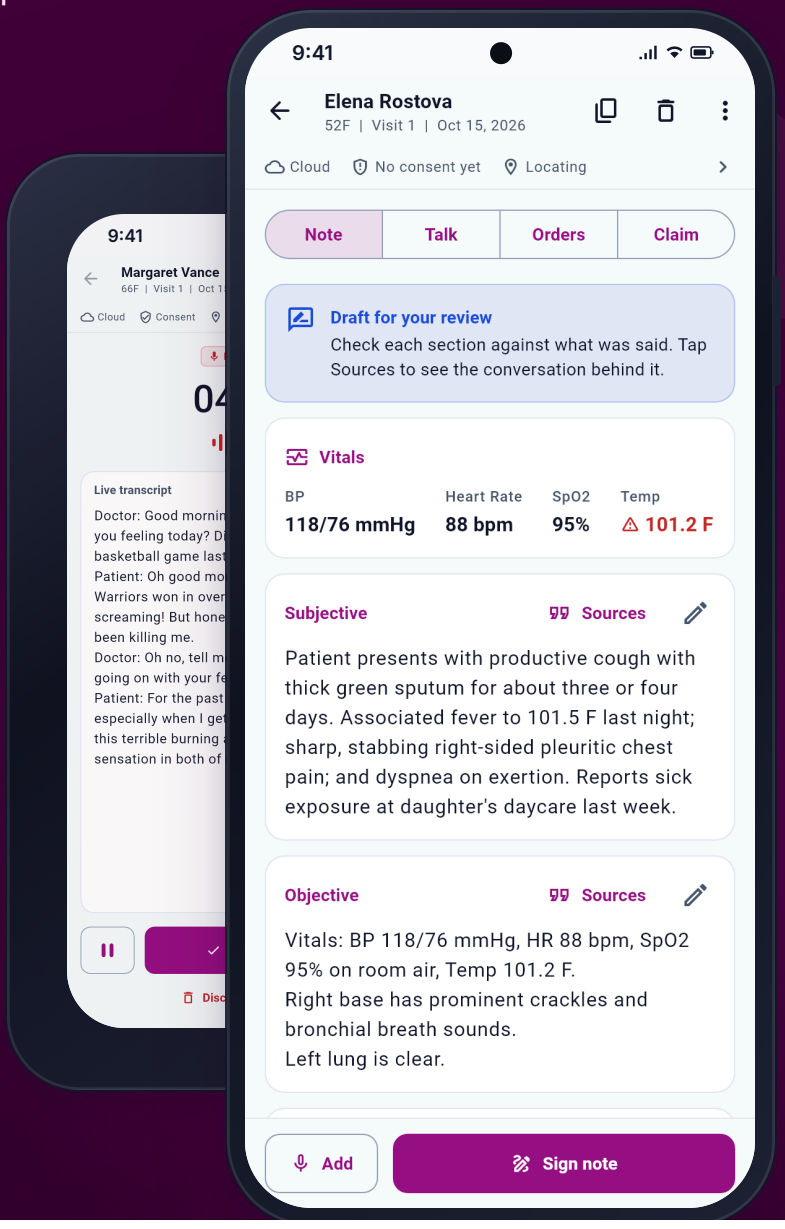
SOLIS

PILOT NOW OPEN

Look at your patient, not the screen.

SOLIS listens to the visit, writes the note, and builds a clean claim, on your phone.



Consent first
A short script in English or Spanish before anything is recorded.

Checked before it goes out
Codes tested against the official CMS code sets for the date of service.

On-device mode
No audio leaves the phone, and it works offline.

REQUEST EARLY ACCESS

solis-health.pages.dev →

The visit ends.
The charting doesn't.

Notes, codes and claims follow physicians home, and every claim still has to get past the payer. The work is necessary. Doing it twice, or at night, is not.

2 hours

of EHR and desk work for **every hour of direct face time** with patients during the clinic day.

Source: Sinsky C, et al. Allocation of Physician Time in Ambulatory Practice. Annals of Internal Medicine. 2016;165:753-760. doi.org/10.7326/M16-0961

1.4 hours

of EHR work **after clinic hours** each weekday, within a 5.9-hour daily EHR total for primary care physicians.

Source: Arndt BG, et al. Tethered to the EHR. Annals of Family Medicine. 2017;15(5):419-426. doi.org/10.1370/afm.2121

1 in 5

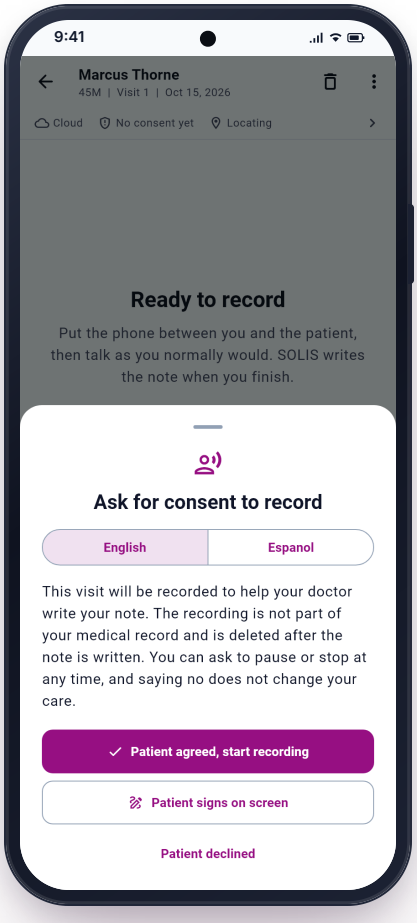
in-network claims denied in 2023 by HealthCare.gov marketplace insurers (20%), and fewer than 1% of denials were appealed.

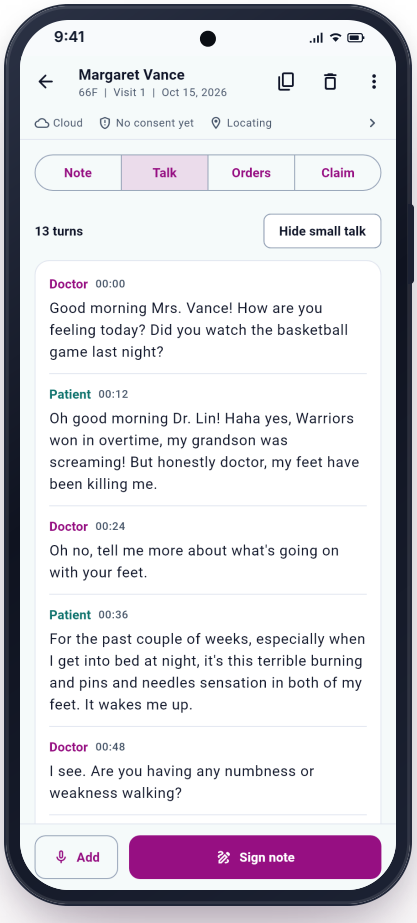
Source: KFF analysis of CMS transparency data, Claims Denials and Appeals in ACA Marketplace Plans in 2023 (updated March 2026). kff.org

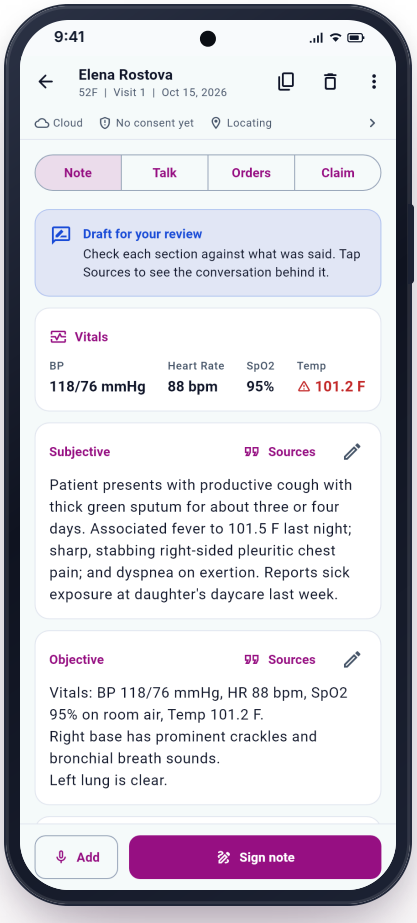
	THE USUAL WAY	WITH SOLIS
	Notes typed from memory after the last patient leaves.	**A concise SOAP draft** from the conversation itself, ready for you to review and sign.
	Codes picked from a cheat sheet, or left for someone else.	**Diagnosis codes from the patient's own words**, each linked to the sentence it came from.
	Errors found when the denial comes back.	**Checks before the claim goes out**, against official CMS code sets for the date of service.
	Insurance details retyped at the front desk.	**The card is read on the phone**, and the photo is deleted after reading.

One visit. Five steps.
One phone on the table.

Put the phone between you and the patient and talk as you normally would. SOLIS handles the paperwork in the background.







01

Consent first

The patient hears a short script in English or Spanish, then agrees out loud or signs on the phone. Time and location are saved with the visit.

02

Capture, voices separated

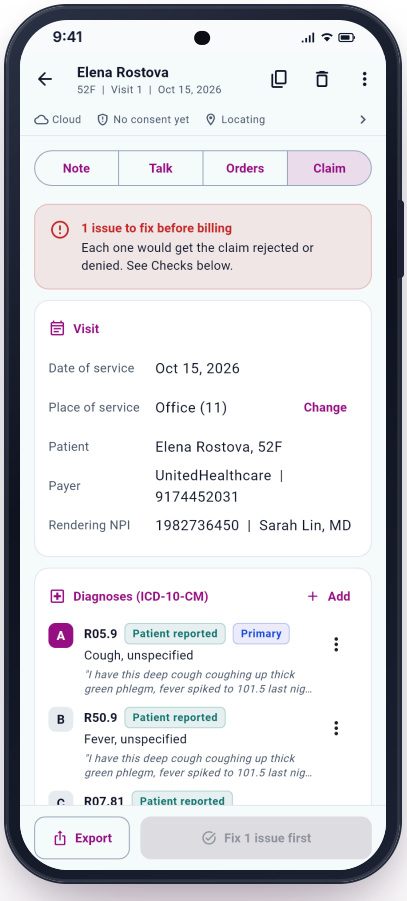
SOLIS tells your voice from the patient's. Choose the on-device engine, or the cloud engine (Deepgram medical model) when your practice turns it on.

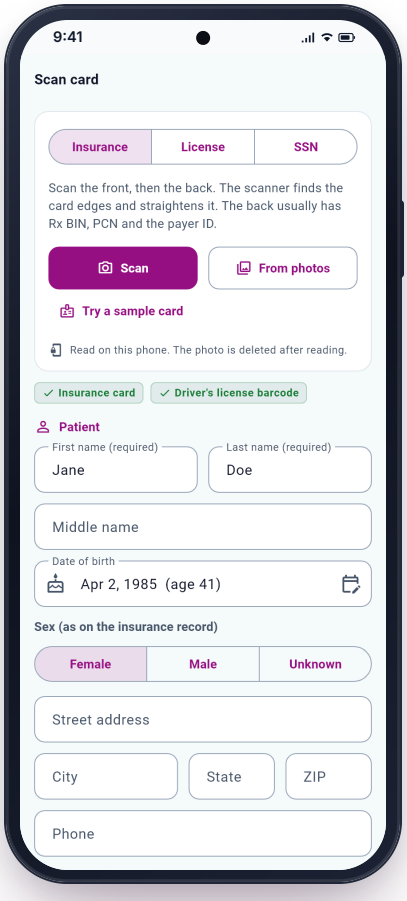
03

The note writes itself

A concise SOAP note. Nothing is invented: no vitals or exam findings that were not said. Tap Sources to see the words behind each section.

From the exam room to a claim you can send.





04

Codes, then checks

Diagnosis codes come from what was said, and uncertain wording waits for your one-tap confirmation. Procedure codes match the diagnosis, with the E/M level set by place of service, new or established patient, and medical decision making.

05

Scan the card, send the claim

Insurance card, driver's license barcode and SSN card are read on the phone with Google ML Kit. Photos are deleted after reading. Only the last 4 SSN digits are kept.

FROM CHECK-IN TO CLAIM STATUS, THROUGH THE STEDI CLEARINGHOUSE

Eligibility
Checked at check-in.

→ **One account**
Check-digit account number, Visit 1, 2, 3 by date.

→ **Claim in one tap**
Sent as an 837P.

→ **Status and export**
Claim status, plus `CMS-1500` data and 837P export.

Coded from the patient's words. Checked against the official rules.

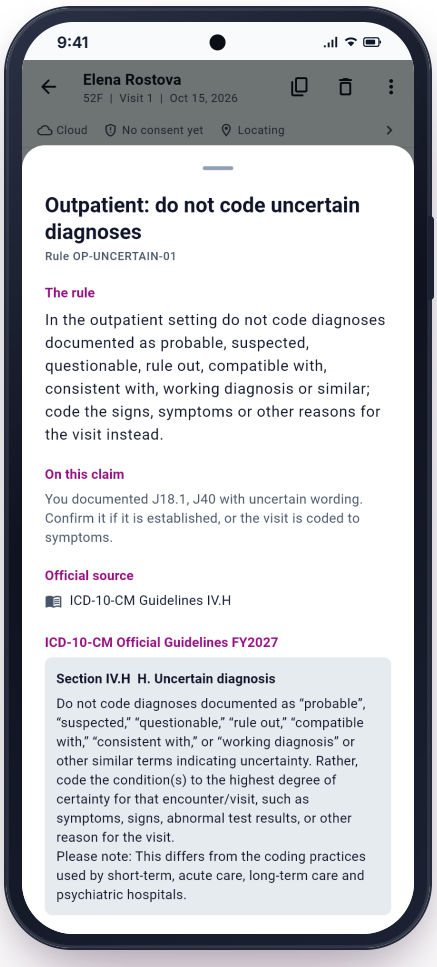
> **"My feet burn at night."**
>
> Coded as a symptom until you confirm a diagnosis, as the official outpatient rule requires (ICD-10-CM guideline IV.H). Confirming takes one tap.

59
of 63 official coding rules fully enforced

4
more need the AMA CPT license, not in the app

Diagnosis codes	`ICD-10-CM FY2026` `FY2027`
HCPCS codes	`HCPCS Oct 2026`
Payment	`2026 Physician Fee Schedule`
Edits	`NCCI MUE` `NCCI add-on`
Age, maternity, manifestation	`Medicare Code Editor v44`
Code structure	`Excludes1` `7th character` `Coding notes`

Code sets in effect on the date of service. Tap **Why** on any check for the rule in plain English, its official citation and guideline text, offline.



What it catches before the payer does

Examples

01
Sex and age conflicts
A code that does not fit the patient.

02
Diagnoses that exclude each other
Excludes1 pairs, flagged before submission.

03
Missing 7th character
Codes that are not yet complete.

04
External cause listed first
Order that a payer would reject.

05
Modifier 25 needed
When a separate E/M service applies.

06
Add-on without its primary
Checked against NCCI add-on edits.

07
Units over the daily limit
Checked against NCCI MUEs.

08
Uncertain diagnoses
Held as symptoms until you confirm.

You decide where the audio goes.

Two engines, one switch. Keep everything on the phone, or let the practice turn on the cloud engine for its medical speech model.

On-device mode

Audio	Stays on the phone
Transcription	On-device, with speaker separation
Note writing	On-device engine
Connection	Works offline
Setup	One-time model download, about 150 MB

Cloud mode

Turned on by	The practice
Transcription	Deepgram medical model
Note writing	Claude
Vendor keys	Held on the SOLIS server
Setup	No model download

Keys never ship in the app

Vendor API keys live on the SOLIS server, not on the phone.

Card photos deleted

Cards are read on the phone, then the photo is deleted. The full SSN is never stored, only the last 4 digits.

Consent on the record

Verbal agreement or an on-screen signature, saved with the time and location of the visit.

A real location

The visit record carries the real street address, never a placeholder.

Built for HIPAA workflows. BAA available.

A Business Associate Agreement (BAA) template is available for practices.

For practices without a billing department down the hall.

Primary care

Office visits coded by new or established patient and medical decision making, with symptoms held until you confirm.

Urgent care

Scan the card at the door, check eligibility at check-in, and send the claim in one tap.

Specialty clinics

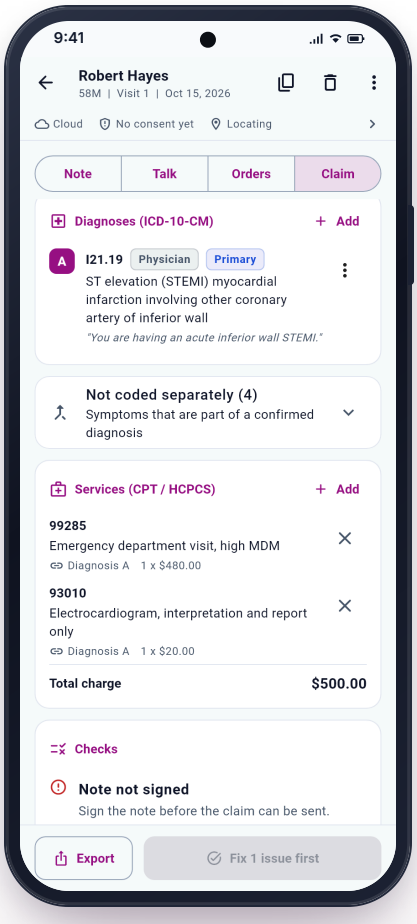
Your specialty and accepted insurers live in your provider profile, and procedure codes link to their diagnoses.

Nursing-home rounds

One account per patient, each visit with its own date and place of service.

Ambulance and EMS

On-device mode works offline, and the visit record keeps the real street address.



Aware of the setting

Place of service shapes the codes: an emergency visit bills the emergency E/M level, and the EKG as interpretation only.

IN VERSION 1.0.0

Platform **Android (iOS not yet released)**	Provider profile **Live NPI registry lookup**	Consent script **English and Spanish**
Clearinghouse **Stedi: eligibility, 837P, status**	Card reading **Insurance, license, SSN card**	Exports **CMS-1500 data and 837P**

06 — GETTING STARTED

Join the pilot.

SOLIS is in its pilot stage. We are inviting physicians and small practices to use it in real visits and tell us what to fix.

01 **Request early access**
Tell us about your practice and the settings you work in.

02 **Set up your provider profile**
Name, NPI (looked up live in the national NPI registry), tax ID, specialty and insurers accepted.

03 **Record your first visit**
Pick on-device or cloud mode, ask for consent, and talk to your patient.

WHAT TO HAVE READY

An Android phone
SOLIS v1.0.0 runs on Android.

Your NPI and tax ID
They go on every claim.

A BAA, if needed
A template is available.

PILOT PROGRAM

Request early access

Website **solis-health.pages.dev**

Scan to visit

SOLIS v1.0.0 for Android. iOS is not yet released. Screens show demo data with fictional patients and practice details. CPT is a registered trademark of the American Medical Association.

SOLIS